

## **\*\*HOSPITAL DISCHARGE SUMMARY\*\***

### **\*\*PATIENT INFORMATION\*\***

Name: Patient 22

Age: 74 years old

Sex: Female

MRN: 000-TEST-22

Admission Date: [DATE]

Discharge Date: [DATE]

Length of Stay: 4 days

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### **\*\*PRIMARY & SECONDARY DIAGNOSES\*\***

- Type 2 Diabetes Mellitus with hyperglycemic hyperosmolar state (HHS)
- Atrial fibrillation (paroxysmal), rate-controlled
- Obesity (BMI 34.2)
- Hypertension
- Hyperlipidemia
- History of diabetic peripheral neuropathy

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### **\*\*CHIEF COMPLAINT & HOSPITAL COURSE\*\***

Patient 22 presented via ED with altered mental status, polyuria, and polydipsia. Initial serum glucose was 687 mg/dL; serum osmolality 319 mOsm/kg. HbA1c on admission: 11.8%. Patient was admitted to ICU for management of hyperglycemic hyperosmolar state with significant dehydration and electrolyte abnormalities (K+ 5.8, Na+ 147).

\*Key interventions:\*

- IV fluid resuscitation with 0.45% NS (total 6L over 48 hrs)
- Insulin drip (per protocol) with transition to subcutaneous regimen by hospital day 2
- Continuous cardiac monitoring (AFib with RVR requiring IV diltiazem × 1 dose)
- Daily basic metabolic panel monitoring; glucose normalized to 220–280 mg/dL by discharge
- Nephrology consulted for CKD stage 3b (eGFR 38 mL/min)
- Endocrinology consult day 2 for insulin titration & education

Patient stabilized without complications. Discharged in stable condition with glucose 185 mg/dL.

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#### \*\*DISCHARGE MEDICATIONS\*\*

| Medication | Dose | Frequency | Duration |
|------------|------|-----------|----------|
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|---------------------------|----------|--------------------|---------|
| Insulin glargine (Lantus) | 30 units | SQ q.d. at bedtime | Ongoing |
|---------------------------|----------|--------------------|---------|

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| Insulin aspart (NovoLog) | 10 units | SQ with meals (sliding scale) | Ongoing |
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| Metformin XR | 1000 mg | PO BID | Ongoing |
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| Metoprolol succinate | 50 mg | PO BID | Ongoing |
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| Diltiazem CD | 240 mg | PO q.d. | Ongoing |
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| Warfarin | 5 mg | PO q.d. (INR target 2–3) | Ongoing |
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| Atorvastatin | 40 mg | PO q.d. | Ongoing |
|--------------|-------|---------|---------|

|            |       |         |         |
|------------|-------|---------|---------|
| Lisinopril | 10 mg | PO q.d. | Ongoing |
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| Furosemide | 20 mg | PO q.d. PRN edema | PRN |
|------------|-------|-------------------|-----|

|         |       |         |         |
|---------|-------|---------|---------|
| Aspirin | 81 mg | PO q.d. | Ongoing |
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## **\*\*MONITORING & FOLLOW-UP\*\***

**\*\*Glucose Monitoring:\*\*** Patient instructed on finger-stick monitoring BID (fasting and bedtime) × 2 weeks, then reassess per endocrinology.

### **\*\*Appointments:\*\***

- Endocrinology: in 1 week (confirm date prior to discharge)
- Primary Care: in 2 weeks
- Cardiology: in 3 weeks (AFib management & Warfarin follow-up)

**\*\*Labs:\*\*** Repeat HbA1c in 3 months; INR check in 3–5 days

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## **\*\*ACTIVITY & RESTRICTIONS\*\***

- Light activity only; gradual increase over 1–2